



Pre-Collection Revenue Cycle Optimization

Dental & Outpatient Chains • CFO / Ownership Briefing

• • • • •

CONFIDENTIAL — INSTITUTIONAL DECK

VitaCoreX LLC • Tampa, FL • EIN 41-4399148
(888) 794-8292 • vitacorexllc.com

© 2026 VitaCoreX LLC. All rights reserved.

EXECUTIVE SUMMARY

Financially-driven operating system

- Patient AR is one of the largest controllable cash drivers in multi-location dental groups.
- Recovery probability decays sharply with aging; every 30 days delay destroys conversion yield.
- A standardized pre-collection layer improves plan adoption, reduces defaults, and limits roll-to-agency.
- Pilot-first deployment produces CFO-grade evidence before scale.

+10–20%

Net cash uplift

25–40%

Agency fee avoidance

90 days

Pilot duration

DSO ↓

Primary lever

AGENDA

Today's discussion

- Market context and why patient AR is now a board-level cash lever
- Patient AR structure and recovery decay curve
- Financial model: DSO compression, working capital release, EBITDA impact
- Operating system: workflow, controls, and governance
- 90-day pilot design, success criteria, and rollout plan
- Commercial model and next steps

THE PROBLEM

Revenue leakage before collections

- Aged patient balances rapidly lose recovery probability.
- Unstructured outreach lowers payment plan adoption and increases early default.
- Early agency escalation creates 25–40% contingency leakage and reputational risk.
- Teams lack a standardized layer that converts balances into documented commitments and autopay.
- Leadership lacks consistent, comparable KPIs across locations.

WHY DENTAL / DSO CHAINS

Fast, measurable financial wins within 90 days

- High patient responsibility share creates a large controllable cash pool.
- Brand sensitivity makes "options-first" pre-collection preferable to external collections.
- Multi-location standardization produces immediate operational leverage.
- RCM and cash outcomes are measurable quickly.

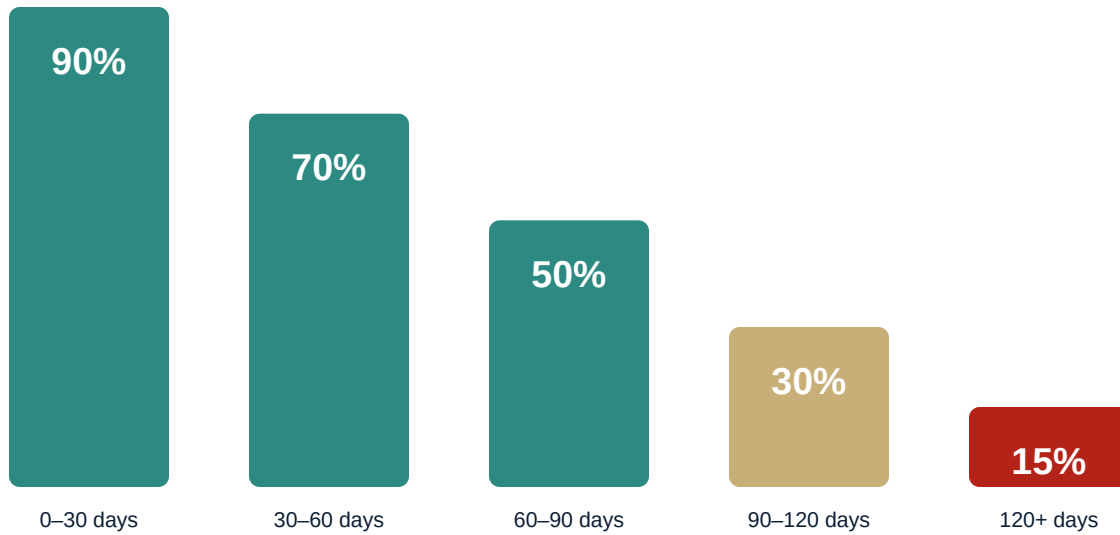
U.S. DENTAL MARKET OVERVIEW

Context for PE-backed DSOs

- U.S. dental services market: \$165B+ annual revenue (industry estimates).
- DSO footprint expanding; consolidation accelerates standardization and KPI discipline.
- Patient responsibility often represents 35–50% of revenue depending on specialty and payer mix.
- Typical AR days: ~35–60 (varies by specialty and process maturity).

RECOVERY PROBABILITY BY AGING

Aging destroys recovery probability



Every 30-day delay materially reduces recovery probability.

HIDDEN CASH IN PATIENT AR

Illustrative exposure in a mid-size dental group

Metric	Example
Annual Revenue	\$40,000,000
Patient Responsibility (40%)	\$16,000,000
Average Patient AR	\$5,200,000
Recovery Loss vs Optimal	10–15%
Cash Leakage	\$520,000 – \$780,000 / year

PATIENT AR COMPOSITION

Why patient balances are the least structured RCM segment

Category	Share
Insurance pending	40–50%
Patient responsibility	25–40%
Self-pay	10–20%

DSO COMPRESSION IMPACT

Working capital release from faster time-to-cash

Metric	Value
DSO reduction	52 → 44 days (-8)
For \$10M revenue practice	Working capital ≈ \$0.22M
At enterprise scale	Meaningful liquidity lever

EBITDA EXPANSION

Incremental recovery converts to EBITDA after costs

Assumption (illustrative)	Value
Annual patient AR portfolio	\$5,000,000
Current recovery (baseline)	25% → \$1,250,000
Pre-collection recovery (target)	38% → \$1,900,000
Incremental recovered cash	+ \$650,000
EBITDA margin example	15%
Incremental EBITDA	\$97,500

ENTERPRISE VALUE IMPACT

Why PE-backed DSOs care about small EBITDA changes

Metric	Example
Incremental EBITDA	\$250,000
DSO / dental multiple	7–9x
Enterprise value impact	\$1.75M – \$2.25M

AGENCY FEE LEAKAGE

Contingency fees reduce margin and enterprise value

Metric	Value
Typical contingency fee	25–40%
Example recovered	\$1.0M
Agency keeps	\$250–\$400K

ROI MODEL

Pilot economics in CFO language

Metric	Value
Implementation cost	\$80K
Incremental recovery (cash)	\$450K
ROI multiple	5.6x
Payback period	3–6 months

90-DAY PILOT FINANCIALS

Data-validated before enterprise rollout

Pilot portfolio	Value
Patient AR in scope	\$2.0M
Expected improvement	+10–15%
Additional cash	\$200K–\$300K
Pilot cost	\$50K
ROI	4–6x

CASE STUDY TEMPLATE

Example dental group (illustrative)

Attribute	Value
Locations	12
Patient AR portfolio	\$4.5M
Autopay enrollment	42%
Recovery rate change	+13%
Agency placements	−28%

CLINIC-LEVEL ROLLOUT

Pilot → standardization → enterprise scale

Phase	Scope	Outcome
1. Pilot	3 clinics 90 days	Validated uplift + playbook
2. Standardize	Policies, scripts, dashboards	Repeatable execution
3. Enterprise rollout	All clinics	Sustained cash velocity + KPI control

STAFF TRAINING AGENDA

Operational discipline without adding headcount

- Payment plan negotiation and options-first framing
- Script training (calls, SMS, email) with cadence caps
- Compliance boundaries and escalation governance
- Exception handling (disputes, hardship, bad contact data)
- CRM workflow: tasks, documentation, audit trail, reporting

CFO ONE-PAGE SUMMARY

Board / ownership language

Area	Detail
Problem	Patient AR aging destroys recovery probability and increases agency leakage
Solution	Pre-collection operating system: standard workflows + controls + reporting.
Impact	+10–20% net recovery; placements ↓; DSO compression; EBITDA uplift.
Approach	90-day pilot to validate before enterprise rollout.

REVENUE CYCLE ARCHITECTURE

Tool-agnostic operating layer

- Tool-agnostic layer on top of PMS/billing exports
- Standardized cases + tasks + documentation
- CFO reporting pack (weekly/monthly)

PRE-COLLECTION WORKFLOW

Sequencing

- Intake → segment → outreach → agreement → autopay → monitor → escalate
- Convert balances into documented commitments before agency fees apply

SEGMENTATION MODEL

Routes accounts by risk profile

- Routes accounts by balance size, age, and contactability
- Standard offers per segment; exceptions reviewed by lead

OFFER CATALOG

Structured payment options

- 3/6/12-month plans with down-payment rules
- Autopay default (opt-out), clear terms and cure periods

AUTOPAY ACTIVATION

Enrollment process

- Enrollment process with clear authorization
- Reduces defaults and increases completion rates

MONITORING ENGINE

Daily exception management

- Daily exception queue (fails, disputes, bad data)
- Cure notices and remediation scripts

ESCALATION LADDER

Governed decision pathway

- Cure → default → acceleration → pre-suit option → agency
- Governance ensures consistent, defensible decisions

CFO KPI DASHBOARD

Executive metrics

- Net yield vs baseline, roll-to-agency %, cohort curves
- Autopay enrollment/completion, exception volumes

GOVERNANCE MODEL

Review cadence

- Weekly ops review; monthly finance review
- Quarterly optimization of offers and scripts

RISK CONTROL FRAMEWORK

Operational safeguards

- Cadence caps, dispute routing, audit trail
- Brand-safe tone and documentation discipline

COMPLIANCE BOUNDARIES

Regulatory alignment

- Informational alignment to FDCPA/Reg F where applicable
- Client retains collections authority unless separately structured

DATA SECURITY & HANDLING

Access and retention

- Least-privilege access; encrypted exports
- Retention policy aligned to compliance program

CRM / DATA ARCHITECTURE

System foundation

- HubSpot/Salesforce-style CRM or RCM tooling
- Single source of truth for commitments + activity

REPORTING PACK (MONTHLY)

Leadership visibility

- Net recovery yield, cohort performance, DSO trends
- Placements avoided, patient experience indicators

90-DAY PILOT DESIGN

Scope and governance

- Scope: clinics, AR cohorts, communication channels
- Success criteria and governance cadence

PILOT SUCCESS METRICS

Measurable outcomes

- Incremental cash, yield uplift, DSO impact
- Placements ↓, default rate ↓, disputes stable

IMPLEMENTATION TIMELINE

Phased deployment

- Week 1–2: diagnostic + setup
- Week 3–6: execution + training
- Week 7–12: optimization + reporting

COMMERCIAL MODEL

Pricing structure

- Implementation + training + optimization
- Pricing depends on clinic count, tools, AR volume

WHAT SUCCESS LOOKS LIKE

Target state

- Higher net recovery with fewer external placements
- Repeatable playbook + CFO-grade reporting

COMMON OBJECTIONS & ANSWERS

Addressing concerns

- "We already use an agency" → reduce placements first, then complement
- "Staff overloaded" → automation + scripts focus on exceptions only

PATIENT EXPERIENCE SAFEGUARDS

Brand protection

- Options-first messaging, quiet hours, dispute/hardship routing
- Reputation protection and review monitoring

WHAT WE NEED FROM YOU

Getting started

- AR export (aging + balances + contact fields)
- Current policy and agency fee structure

NEXT STEPS

Getting started

- 01 15-minute fit call
- 02 NDA + AR export request
- 03 Pilot scope confirmation and launch

vitacorexllc.com • (888) 794-8292

APPENDIX: KPI DEFINITIONS

- Net recovery, roll-to-agency, completion rate, cash velocity

VitaCoreX LLC is not a law firm and does not provide legal representation.

APPENDIX: OUTREACH CADENCE

- Day 1 notice; Day 7 plan; Day 14 reminder; Day 21 final notice

VitaCoreX LLC is not a law firm and does not provide legal representation.

APPENDIX: ROLES & RESPONSIBILITIES

- Finance: KPI targets; Ops/RCM: execution; Billing: disputes; Agency: last-mile

VitaCoreX LLC is not a law firm and does not provide legal representation.